

VESYCA® FILM COATED TABLET 150MG

Ranitidine is a potent H₂-antagonist. In vivo and in vitro studies have shown that it is a competitive H₂-antagonist without significant activity at the histamine H₁-receptors or at the cholinergic receptors sites. Ranitidine is an aminomethylfuran derivative; the chloride salt is chemically known as N[2][{[5[(dimethylamino)methyl]-2-furanyl]methyl}thio]ethyl]-N-methyl-2-nitro-1,1-ethenediamine, hydrochloride.

Ingredient(s):
Each tablet contains:
Ranitidine (as HCl) 150mg

Pharmacology (Summary of Pharmacokinetics and Pharmacodynamics):

- a) Ranitidine is a potent H₂ antagonist. It antagonizes the effects of histamine in vitro upon isolated guinea pig atrium and rat uterine horn which possess H₂-receptors, but not guinea pig ileum which has H₁-receptors. There is displacement to the right of the histamine dose-response curve indicating that ranitidine is a competitive antagonist. Ranitidine is about 5 times more potent than cimetidine in these tests on a molar basis. It is a potent inhibitor of gastric secretion induced in response to histamine, pentagastrin, bethanechol and test meals.
- b) Ranitidine is a competitive, reversible inhibitor of the action of histamine at the H₂-receptor site, including receptors on the gastric cells. It does not lower serum calcium in hypercalcemic states.
- c) Ranitidine is readily absorbed from the gastrointestinal tract with peak concentrations in plasma occurring about 2 hours after oral administration. The elimination half-life from plasma is around 2.5 ~ 3 hours. A small proportion of ranitidine is metabolized in the liver and most is excreted unchanged in the urine. It does not readily cross the blood brain barrier.
- d) Ranitidine has no effects on serum gonadotropin, TSH, or GH. It may cause impairment of vasopressin release.
- e) No change in cortisol, aldosterone, androgen, or estrogen levels have been observed. Ranitidine has no antiandrogenic action.

Indication(s):
Duodenal ulcers, benign gastric ulcers, active peptic ulcer, reflux esophagitis, Zollinger-Ellison Syndrome prophylaxis of gastrointestinal haemorrhage from stress ulceration in seriously ill patient, prophylaxis of recurrent haemorrhage in patients with bleeding peptic ulcers before general anaesthesia in patients considered to be at risk of acid aspiration, particularly obstetric patients during labour.

Dosage and Administration:
Adults:
In the management of duodenal and gastric ulcers a single daily dose of 300mg by mouth at bedtime is suggested as an alternative to one 150mg tablet twice daily, taken in the morning and before retiring. It is not necessary to time the dose in relation to meals. In most cases of duodenal ulcers, benign gastric ulcer and post-operative ulcer, healing occurs in 4 weeks. In the small number of patients whose ulcers may not have fully healed, these are likely to respond to a further 4 weeks course of treatment. Maintenance treatment at a reduced dosage of one 150mg tablet at bedtime is recommended for patients who have responded to short-term therapy, particularly those with a history of recurrent ulcer.
In the management of reflux oesophagitis, the recommended course of treatment is one 150mg tablet twice daily for up to 8 weeks.
In patients with Zollinger-Ellison Syndrome, the starting dose is 150mg three times daily and this may be increased, as necessary. Doses should be adjusted to individual patients' needs. Doses up to 6g/day have been well tolerated.

Prophylaxis of hemorrhage from stress ulceration in seriously ill patients or prophylaxis of recurrent hemorrhage in patients bleeding from peptic ulceration who are currently managed by i. v. ranitidine: an oral dose of 150mg twice daily may be substituted for the injection once oral feeding commences.
Prophylaxis of acid aspiration syndrome (AAS) : 150 mg in the evening prior to anesthesia induction is recommended, however, 150mg before anesthesia is also effective. For the prevention of AAS in pre-partum patients who elect for anesthesia, 150mg every 6 hours may be employed, but if general anesthesia is warranted, a nonparticulate oral antacid (e.g. sodium citrate) could supplement ranitidine therapy. In an emergency situation, the use of alkalis, antacids, and meticulous anesthetic technique is still necessary as ranitidine does not affect the pH and volume of the existing gastric content.
Patient with impaired renal function:
For patient with severe renal impairment (creatinine clearance < 50ml/min), the recommended daily dosage is 150mg.

Children :
Experience with ranitidine in children is limited and such use has not been fully evaluated in clinical studies. It has however been used successfully in children aged 8 ~ 18 years in doses up to 150mg twice daily without adverse effect.

Contraindication(s):
Contraindicated in patients who have shown hypersensitivity to the product.

Side Effect(s) / Adverse Reaction(s):
Adverse reactions to ranitidine are generally infrequent and are usually reversible following a reduction of dosage or withdrawal of therapy. The commonest side effects reported are diarrhea, dizziness, tiredness and rashes.
Reversible confusional states, especially in the elderly or in seriously ill patients such as those with renal failure have occasionally occurred.
Unlike cimetidine, ranitidine has little or no antiandrogenic effect and the incidence of gynecomastia and impotence in patients treated with higher doses of ranitidine has been reported not to differ from that encountered in the general population.
Other adverse effects which have been reported rarely are allergic reactions, arthralgia and myalgia, blood disorders including agranulocytosis or granulocytopenia and thrombocytopenia, headache, hepatotoxicity and pancreatitis. Hypersensitivity reactions such as urticaria, angioneurotic oedema , fever, bronchospasm, hypotension and anaphylactic shock have been very rare following administration of ranitidine.

- Precaution(s) / Warning(s):**
- a) Symptomatic response to ranitidine therapy does not preclude the presence of gastric malignancy. The possibility of malignancy should be excluded before commencement of therapy in patients with gastric ulcer.
 - b) Since ranitidine is excreted primarily through the kidney, dosage should be adjusted in patients with impaired renal function.
 - c) Caution should be observed in patients with hepatic dysfunction since it is metabolized in the liver.
 - d) Ranitidine may precipitate acute porphyric attacks, therefore it should be avoided in patients with a history of acute porphyria.

Used in pregnancy and lactation:
The safety of ranitidine in pregnancy has not been established. If the administration of Vesyc is considered to be necessary, its use requires that the potential benefits be weighed against possible hazards to the patient and to the fetus.
Ranitidine is secreted in breast milk in lactating mothers, but the clinical significance of this has not been fully evaluated. It should therefore be used by nursing mothers only if considered essential.

Drug Interaction:
Antacids appear to reduce the gastrointestinal absorption of ranitidine, but the mechanism of which this occurs is unknown.

Symptoms and Treatment for Overdosage, and Antidote(s):
There is no specific antidote for ranitidine overdose; clinical monitoring and symptomatic and supportive therapies should be employed. If necessary, the drug may be removed from the plasma by haemodialysis.

Shelf-Life:
2 years from the date of manufacture.

Storage Condition(s):
Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):
A yellow color film coated tablet, one side impressed with a mark "  "
Strip packing of 10's x 10 and 10's x 50.


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